جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : قدريه السيد احمد العوفي
المحافظة : القليوبيه **الحي / المدينة :** الخانكه **العنوان :** زعبل
رقم البطاقة : 25606281400681 **رقم التليفون :** 01004863827 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 28-06-1956 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: قدريه السيد احمد العوفي
National ID: 25606281400681

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name : Qadriya El-Sayed Ahmed Al-Oufi
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 18-05-2026 12:04
Medical No. : 377711
Ins.Company : تعاقد مستشفيات

NILE DIAGNOSTIC CENTER

Tc-99m MDP Whole-Body Skeletal Scintigraphy

- A 70-year-old female patient with a known history of breast cancer, referred for follow-up evaluation and comparison with prior imaging.
- Documented history of orthopedic intervention in the left lower limb involving internal fixation (intramedullary nail) of the left tibia in Feb. 2024.

Procedure: Anterior and posterior whole-body planar scans acquired approximately 2 hours post-intravenous injection of the tracer (Tc-99m MDP).

Findings – Comparison with Prior Study (Dated 29/10/2025):

- The previously described active foci involving the right humeral head, bilateral ribs, lower lumbar vertebrae, sacrum, bilateral sacroiliac regions, and right acetabulum are visualized. They demonstrate stable tracer intensity and spatial distribution without evidence of progression.
- No new independent focal areas of abnormal increased radiotracer uptake are identified throughout the remainder of the axial or appendicular skeleton.
- Persistent, prominent focal accumulation of radiotracer is noted at the proximal aspect of the left tibia. This site shows a linear structural pattern correlating directly with the patient's history of a left tibial intramedullary nail. The uptake is consistent with expected long-term post-surgical osseous remodeling and localized biomechanical stress.
- Diffuse, mild-to-moderate heterogeneous uptake continues to be noted along the spinal column and large peripheral joints, most pronounced symmetrically in both knee joints, consistent with prominent bilateral osteoarthritis.

Impression

- In comparison with the previous skeletal scintigraphy dated 29/10/2025, the known metastatic osseous lesions show a more or less stationary course.
- No new active focal lesions depicted.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Name: قدريه سيد احمد يسريه	
MR. : 5204	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

POSTM RT BREAST CANCER MET TO BONE MED LN LUNG IDC2 ER8/8 PR7/8 HER2-VE DIAGNOSED IN 2024 REC AROMASIN TILL 2/2025 DP LEFT TIBIAL PATHOLOGICAL FRACTURE INTERNAL FIXATION ST FASOLDEX IBRANCE FROM5/2025 (PT REC PALL RTH TO L5 SIJ).BS 10/2025 SD FOR NEW CTS BS FOR FASOLDEX IBRANCE 125

2/5/2026new CT CAP without comparison -- reports compared, SD, pending bone scan, for ttt renewal for neck US (22 mm thyroid lesion seen on ct) today for zometa calcium and creat done 15 days ago .. accepted

ibrance 125 + fasoladex 500 next visit :10/8/2026

cts 5/2026 sd bs 5/2026 on exam rt retroaleolar mass 3x3 nipple retraction for zometa fasoldex ibrance 125 for new cts next month

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

ne:	Qadreya Elsayed Ahmed
Date:	2026-05-02
Referred by:	Prof.Dr. CAPwithC
Hospital:	Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA with no available previous films for comparison.

I- CHEST:

- Right breast lesion is seen measuring 25 x 20 mm for sono-mammography.
- Multiple bilateral scattered pulmonary nodules largest measuring 7 mm at the left upper lobe.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.
- Bilateral small elastofibroma dorsi.
- Left thyroid lobe hypodense nodule measuring 22 mm in diameter.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple axial and appendicular skeleton sclerotic bone lesions.

Opinion:

- **Multiple bilateral scattered pulmonary nodules.**
- **Multiple axial and appendicular skeleton sclerotic bone lesions likely metastatic.**

❖ *Dr.sherif Wagdy*

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Qadriya El-Sayed Ahmed Al-Ouli
Branch : Obour
Ref.Physician : Prof ; Goustav Roussey Egypt

Date : 18-05-2026 12:04
Medical No. : 377011
Ins.Company : كعلف مستشفيات

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Impression

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- No new active focal lesions depicted.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

ne: Qadreya Elsayed Ahmed
Date: 2026-05-02
Referred by: Prof.Dr. CAPwithC
Hospital: Goustave Roussey International Egypt

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- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple axial and appendicular skeleton sclerotic bone lesions.

Opition:

- **Multiple bilateral scattered pulmonary nodules.**
- **Multiple axial and appendicular skeleton sclerotic bone lesions likely metastatic.**

Dr.sherif Wagd

CT Scan
[[DOCTYPE:CT_SCAN]]

MDT form

1- Patient ID:		3- Medical number:	92214595		
2- Name:	[redacted]	Age:	Female	DOB:	1951-2024
File number:	[redacted]				

Referring doctor	[redacted]	Phone	[redacted]	BMI	REASON OF MDT
Referring clinic		NRS Score		Gross	Date of MDT
Performance status		Menopausal status			

Clinical data

Rt BREAST LEFTQUARTOL [illegible]	Mets Workup ct cap: PR in rt breast mass , rt axillary LN multiple pulmonary nodules largest 1.3 x 1 cm few mediastinal LN largest 2.2 X1.4 CM L1 bony lesion BONE SCAN BONY METS

ER	PR	HER 2 (IHC)	SISH	KI67
8/8	7/8	0		30

T	N	M
CT4B	2	1 LUNG ,BONE

Specimammography

PATIENT HISTORY

started 4 month ago with rt breast lumps and dorsal pain
bilateral SM : revealed rt breast mass
IMBC to bone, lung , LN (mediastinal)
started 1st line aromasine 7/2024 till now
recently developed lt tibial (pathological spontaneous fracture)
2/2025 -->orthopedic surgery (nail) 8/3/2025
ct cap 1/2025: PR in rt breast, rta xilla, multiple bilateral pulmonary nodules, mediastinal LN
S[illegible]n bone
bone scan 2/2025: newly dveloped multiple bony sites lesion PD

MDT OUTCOME & PLAN

for ibrance - fasoldex
zometa q / 3 month

SIGNATURE

MEDICAL ONCOLOGY [redacted] RADIOLOGY

[[DOCTYPE:MDT_FORM]]

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Name : Kadreya Embaby Senosy Abd Al Kader	**Date : 25-08-2025 01:36**
Branch : Portable	**Medical No. : 32563**
Ref.Physician Prof :	**Ins.Company :**

MULTI-SLICE NON-CONTRAST CT SCAN OF THE CHEST WITH RECONSTRUCTION IMAGES REVEALED:

FINDINGS:

- Known case of myeloma, porta cath is noted
- Left lower lobar posterior segment subpleural consolidation with air bronchogram and underlying pleural thickening.
- Bilateral lower lobar fibrotic strands likely post inflammatory sequel.
- No sizeable hilar or mediastinal lymphadenopathy.
- No evidence of pleural or pericardial effusion.
- No gross cardiac abnormality.
- Normal appearance of the chest wall.

OPINION:

Left lower lobar consolidation with air bronchogram likely bronchopneumonia.
Bilateral lower lobar fibrotic strands likely post previous inflammatory sequel.
Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	:الاسكاء / فدوية السيد احمد الحوفي	Request No.	:13002145
Patient No.	:25606281400681	Request Date	:29/05/2024
Path No.	:1807/24	Verification Date	:23/06/2024
Sex	:Female	Financial Category	:برنامج دار السلام - مباشرة
Age	:68 Y	Department	:Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	:Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast retro-areolar mass.
Nature of Specimen: Tru-cut biopsy, collected at 28/05/2024.

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Results:
IPT (Positive control showed apropriate reactivity):
ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
PR: Strong nuclear reaction in 80% of tumor cells (score 7/8).
Her2: No membrane reaction in the tumor cells (score 0).
Ki67: 30% nuclear reaction of tumor cells.

Conclusion:
Right breast retro-areolar mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:
ER: Positive (score 8/8).
PR: Positive (score 7/8).
Her2: Negative (score 0).
Ki67: Positive in 30% of tumor cells.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1

Verified by :

Room No. (4) - First Floor Building No. 95 - 6th Corniche El Nile - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / الدريه السيد احمد العوفى :	Request No.	:13008458
Patient No.	:25606281400681	Request Date	:09/03/2025
Path. No.	:937/25	Verification Date	:08/04/2025
Sex	:Female	Financial Category	: اورام دار السلام - مباشره
Age	:68 Y	Department	:Out Patient
Referreing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Known case of breast cancer
Nature of the specimen: Bone marrow biopsy
Gross Examination:
Fragmented grayish white tissue core measured 2 cm long. Totally submitted.
Microscopic examination:

- Adequate: yes
- Cellularity: Marked hypocellularity
- Myeloid/erythroid ratio: within average
- Alypical cells infiltration: No
- Fibrosis: No
- Additional pathologic findings:

CK7: Negative.

Diagnosis:
BMB: Marked Hypocellular marrow. Negative for neoplastic infiltrate.

Dr. Marwa Mahmoud
Dr. Menna Nabil

Verified by :
Dr.Marwa mahmoud

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	الأبلاء / فدوية السيد احمد العوفى	Request No.	13002193
Patient No.	25606281400651	Request Date	28/05/2024
Path No.	1807/24	Verification Date	23/06/2024
Sex	Female	Financial Category	اورام دار السلام - مدفوع
Age	68 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining); intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system according to Reference: ASCO/CAP HER2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is [illegible] and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Marwa Mahmoud.
Dr. Sagda Elkady.

Verified by :

CAP [illegible] Nile Corniche - Misr El Qadima - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية التخصصية
مستشفى أورام دار السلام
الأشعة

Patient Name: Kadreya Al Sayed Ahmed Aloofi	Date : 08/01/2025
Exam : C.T CHEST	Age : 68 Year(s)
Medical ID: 1953/2024	Pat. ID: 49604

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

- Reported case of right breast cancer. As compared to last study dated 24/6/2024

I Chest:-

- Slight regressive stationary course regarding the size of The right breast defined soft tissue lesion , currenly méasures about 2.6x2.3 cm compoared to about 3x2.7 cm.
- Regressive course regarding the size and number of the enlarged axillary lymph nodes,the largest one mearsures 11x10mm compared to 21x12mm at the previous study.
- Regressive course regarding the size and number of the bilateral scatterd variable size and shape well defined pulmonary nodules, the largest seen at the posterior basal segment of left lower lobe now measuring about 8x5mm compared to 14x10 mm at the last study.
- Regressive course regarding to the size of enlarged few mediastinal lymph nodes .the largest left paratracheal measuring 15x8mm compared to 22x14 mm at the last study.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities on non contrast CT
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.

CT Scan
[[DOCTYPE:CT_SCAN]]